

Medication	Dose	Notes	Side effects
Lamotrigine	• Starting dose: - Week 1 and 2: 25mg daily - Week 3 and 4: 25mg 12 hourly - Week 5: 25mg in the morning and 50mg at night - Week 6: 50mg 12 hourly - Week 7 onwards: increase by 50mg every 2 weeks until controlled • Usual maintenance dose: 50-100mg 12 hourly (or 100-200mg daily) • Maximum dose: 250mg 12 hourly • If switching from sodium valproate: - Continue sodium valproate while starting lamotrigine. - Titrate lamotrigine up slowly: - Week 1 and 2: 25mg on alternate days - Week 3 and 4: 25mg daily - Week 5: 25mg 12 hourly - Week 6: 25mg in the morning and 50mg at night - Week 7: 50mg 12 hourly - Once on full dose of lamotrigine, slowly reduce sodium valproate dose over 4-6 weeks until stopped.	• Preferred anticonvulsant if on ART. • No significant interactions with dolutegravir. • If on lopinavir/ritonavir: doctor to double the dose of lamotrigine. • May also interact with paracetamol, rifampicin, other anticonvulsants, oral contraceptive: check SAMF or discuss with MIC ↗ 178. • If known liver or kidney disease, discuss with specialist. • If lamotrigine not suitable or not tolerated, refer. • If treatment is interrupted for > 1 week, titrate up again using starting dose.	• Urgent: rash ↗ 73 • Self-limiting: nausea, vomiting, blurred or double vision, dizziness, drowsiness, insomnia, fatigue
Carbamazepine	• Starting dose: 100mg 12 hourly for 1 week, then 200mg 12 hourly for 1 week. If needed, increase every week by 100-200mg/day. • Usual maintenance dose: 300-600mg 12 hourly • Maximum dose: 600mg 12 hourly	• Avoid if on/need ART. • May interact with dolutegravir, isoniazid, rifampicin, warfarin, fluoxetine, amitriptyline, theophylline, other anticonvulsants, oral/subdermal contraceptive: check SAMF or discuss with MIC hotline ↗ 178.	• Urgent: rash ↗ 73 • Self-limiting: drowsiness, dry mouth, dizziness, nausea
Phenytoin	• Starting dose: 200mg at night (this is equivalent to 4.5–5mg/kg lean body mass daily). If needed, increase up to 300mg daily (or 150mg 12 hourly). • Maximum dose: 300mg daily	• Only use if already well controlled on phenytoin. • Avoid if a woman or on/need ART. • May interact with isoniazid, rifampicin, warfarin, fluoxetine, fluconazole, theophylline, folate, other anticonvulsants, oral/subdermal contraceptive: check SAMF or discuss with MIC hotline ↗ 178. • If on > 300mg daily, monitor drug levels regularly. Take trough level 2-3 days after initial dose adjustment (timing of trough levels is not critical if on extended release formulation). If needed, adjust dosing according to result. Repeat level after 5-8 days since initial dose change. Repeat weekly until stable. Once stable, monitor levels at 3-12 month intervals.	• Urgent: - Rash ↗ 73 - If unsteady on feet, blurred/double vision or slurring, doctor to check phenytoin level for toxicity. If doctor not available, refer same day. • Self-limiting: drowsiness • Other: large gums; facial hair/course features in women: switch medication.

Review the patient with epilepsy

- If no further fits, review 6 monthly.
- If still fitting, doctor to review monthly until fits stop.
- Refer if any of:
 - Newly diagnosed for CT scan
 - Seizures other than generalised tonic-clonic seizures (e.g. absence and focal seizures)
 - Fits increasing in frequency or changing in type
 - No fits for ≥ 2 years, for possible treatment withdrawal
 - Patient has switched anticonvulsant once and is adherent but still fitting after 4 weeks on maximum dose of second anticonvulsant.